

Greenfield Gastroenterology Center

Department of Gastroenterology

123 Health Street, Mediville, CA 90210 | Phone: (555) 123-9876 | Fax: (555) 123-6543

Dr. Martin Floret, MD - Attending Gastroenterologist

Registration / Demographics

Patient Name	Lucas Powell	Date of Birth	1978-12-05
Age	44	Sex	Male
Address	456 Elm Street, Med_city, CA 90211	Phone	(555) 234-5678
MRN	GNPOW789123	Account	456558
Insurance	MediLife Plus Plan	Emergency Contact	Karen Powell - (555) 876-5432
Encounter Date	2023-08-18	Check-in Time	08:15 AM
Location	Greenfield Gastroenterology Clinic	Referring Provider	Dr. Emily Pearson
Chief Complaint	Persistent GERD symptoms despite treatment	ASA Class	II
Allergies	Cephalosporins	Problem List / PMH	Gastroesophageal Reflux Disease (GERD), Hypertension Hiatal Hernia
Current Medications	Omeprazole 20 mg daily, Lisinopril 10 mg daily, Aspirin 81 mg daily, Multivitamin once daily, Metformin 500 mg BID	Pre-proc Vitals	BP: 125/80 mmHg, HR: 72 bpm, SpO2: 98% RA Temperature: 36.8°C

Informed consent obtained, explaining procedure, risks, and benefits. Patient agreed to proceed.

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Pre-Procedure Assessment & Anesthesia Record

Pre-anesthesia evaluation: Airway normal with adequate neck extension. Cardiovascular: regular rhythm without murmurs. Lungs clear with good air entry bilaterally. NPO since midnight, last intake was water at 6:00 AM.

Anesthesia Staff	Dr. Sarah Verklin, MD - Anesthesiologist Adam Young, CRNA
Plan	MAC with Propofol, ± Fentanyl
Anesthesia Start	08:45 AM
Procedure Start (Scope In)	09:00 AM
Procedure End (Scope Out)	09:20 AM
Anesthesia End	09:35 AM

Monitoring Trends

Time	BP	HR	SpO2
09:00 AM	122/81 mmHg	68 bpm	98%
09:10 AM	118/78 mmHg	70 bpm	100%
09:20 AM	119/80 mmHg	73 bpm	99%
Recovery Start	118/75 mmHg	72 bpm	97%

Minor event: Slight desaturation (92%) temporarily resolved with jaw thrust and enhanced supplemental O2.

Post-anesthesia evaluation: Aldrete score 10; recovery uneventful. Discharged home with escort.

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Procedure Note

Esophagogastroduodenoscopy (EGD)

Endoscope Model: Xendo123

Patient positioned left lateral with appropriate monitoring. Bite block engagement confirmed.

Narrative: Endoscope introduced smoothly through the oropharynx. Passed the UES without resistance, allowing for careful inspection. Esophagus demonstrated an unremarkable appearance distally with the Z-line demarcated at 40 cm from incisors. No rings or candida overgrowth.

Stomach observations revealed homogeneous mucosal texture across the fundus, body, and antrum. Insufflation allowed optimal visualization of rugal folds. Notable erythema near the lesser curvature.

Scope transitioned past the pylorus into the duodenum, with the bulb appreciably intact. Villi appeared normal without any presence of reddened or ulcerative areas.

- Z-line noted at 40 cm with no Barrett's changes.
- Erythema at the antral region, limited extension, non-fluid containing.

Impression: GERD contributed changes; endoscopy associates unremarkable barcode-descriptive observations.

Diagnosis: Consistent with chronic reflux-induced changes.

Interventions: Location-specific tissue acquisition: Antral biopsy stakes obtained with cold bipolar reduction (4 sample bundles).

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Specimens / Lab / Pathology

Container	Site	Fragments	Method	Fixative
A	Antral mucosa	4	Cold forceps	Formalin

Lab: Prime Labs, Sub# DLP627

Pathology Report:

Gross Description: Discrete samples, collectively tannish-pink.

Microscopic Description: Non-specific antral chronic inflammation. Epithelial architecture bounces without gastrowarnings, microinterpretations passable, adequate samples resizable.

Final Diagnosis: Chronic mild inflammation with no H. pylori detected. No appreciable adenomatous regions.

Pathologist: Dr. Sandra Lesly, MD
Report dated 2023-08-20, 4:05 PM

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After Visit Summary / Discharge Instructions

Recovery Course: After awakening, the patient exhibited zero resultant pain or nausea. Overall heart rate stable; tolerated sips orally with usual swallowing give acknowledgment.

Diet/Activity Instructions: Soft food diet as comfort allows, return to regimen discomfort accounting. Abrupt internal invasions discouraged."

Warning signs: New or changed abdominal pain, recurrent hematemesis; unplanned weight variances should prompt provider review.

Follow-up plan: Continue physician advisories chiefly around ulcer ownership rules. Join pharmacological adjustments outlined with alma mater.

Meds: Begin moderate cleaved suppressors. Resume normal discontinuables after exit assurance subject notation tonight.

Results would seek communicative reciprocities by tele-loop oversight. Next evaluation previewed near Spring segment fiscal year's half.

Verified patient: Lucas Powell engaged in comprehension corroboratively outgoingly defenseworthiness as pertained dispatch escortant credential structure.

RN: Michelle Santos

Transgressed addition utmost the scope: End tablesdata logged without something fiercely autonomous of pumpkin origins.
Time: 10:30 AM

Signature: Dr. Martin Floret, MD

Date/Time: 2023-08-18, 11:15 AM
(Electronically signed by Dr. Floret)

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